GENDER-BASED VIOLENCE
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CHAPTER 10
Gender-Based Violence Against Children and Adolescents

Child protection, safe disclosure, prevention, survivor-centred responses and pathways to help in Rwanda

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For general learning and trained community educators. Not a clinical, forensic or legal protocol. Specialist review is required before publication.

Who this chapter is for: Parents and caregivers, adolescents, educators, community workers, health and social-service personnel, and people designing safe child-protection learning resources.

How to use: Read the lessons in order. Case studies are fictional and non-graphic. Facilitators should never invite children to disclose personal abuse in a public classroom or group exercise.

Immediate concern: If a child may be in danger, prioritize immediate protection and contact appropriate local emergency, child-protection or healthcare services. Do not wait to finish this chapter.

Learning outcomes

- Explain child- and adolescent-related gender-based violence without blaming children.
- Recognize common forms and warning signs while understanding that signs alone do not prove abuse.
- Describe age-appropriate body safety, boundaries, and the limits of apparent consent where power or age prevents meaningful agreement.
- Respond to a disclosure without interrogation, leading questions or promises of absolute secrecy.
- Identify time-sensitive healthcare needs and safe, Rwanda-relevant referral options.
- Apply prevention principles across households, schools, community services and digital spaces.

01 | Understanding violence against children and adolescents

Gender-based violence (GBV) includes harmful acts rooted in gender-related power differences, discrimination, or expectations. Children and adolescents can experience sexual abuse, harassment, exploitation, forced or coerced relationships, emotional intimidation, and violence in the home, school or digital environment. Other forms of violence against children may not be gender-based; nevertheless, all children need protection.

The term child generally refers to a person below 18 years in international child-rights frameworks. Adolescence overlaps with childhood; young people have developing decision-making abilities and increasing autonomy, while adults and institutions retain responsibilities to protect them. Protective responses should respect a young person’s dignity, voice, development and safety.

GBV can affect girls and boys, including children with disabilities and children who are socially excluded. Girls may face distinctive gendered risks, but no child should be overlooked because of sex, gender, background, disability or perceived identity.

02 | What forms of GBV can affect children?

Form	Explanation and example
Sexual abuse or exploitation	An adult or other person uses coercion, manipulation or a position of power to involve a child in sexual activity; exchange of school benefits or gifts for sexual access.
Sexual harassment	Unwelcome sexual comments, sexualized messages, unwanted touching or intimidation at school or elsewhere.
Forced or coerced union	Pressuring a child into a marriage or relationship they cannot freely choose.
Psychological abuse	Gendered humiliation, threats, isolation or coercive control that undermines safety and confidence.
Technology-facilitated abuse	Sexual extortion, grooming, threats to share private images, unwanted sexual messages or persistent tracking.
Gender-related exclusion	Restrictions on schooling or opportunities enforced through harmful stereotypes, threats or violence.

These categories may overlap. A teacher who uses grades to demand a sexual favor combines abuse of authority, exploitation and harassment. The label matters less than recognizing harm and connecting the child with safe help.

03 | Where abuse may occur

Risk can arise within families and extended households, schools and boarding settings, sports or religious activities, workplaces involving young people, transportation, camps, service institutions and online spaces. Abuse may be perpetrated by someone known and trusted, not only by strangers. Familiarity and social status must never replace safeguarding checks.

04 | Power, dependence and age-appropriate consent

Children deserve to have their views heard and to understand choices affecting them. Yet a child’s apparent compliance, silence, receipt of gifts, or previous friendliness must not be treated as consent to sexual abuse. Significant age, authority and dependence may make refusal unsafe or meaningful agreement impossible. Applicable national law determines legal age thresholds and offences; this educational chapter does not give an exhaustive statement of Rwandan legal requirements.

For healthy daily interactions, children can learn that they may express discomfort, say no to unwanted contact, ask questions before sharing personal information, and seek help from safe

adults. Such lessons never transfer responsibility for preventing abuse from adults to children.

05 | Grooming and manipulation

Grooming involves gaining a child's trust, access, secrecy or dependence to facilitate exploitation. It may involve excessive gifts, special treatment, isolating a young person, requesting private conversations, testing boundaries or using fear and threats. None of these behaviors alone proves grooming; patterns and context matter.

Adults should be alert to boundary violations without teaching that every caring adult is dangerous. A useful school or community rule is that safeguarding relationships remain transparent, appropriately supervised and accountable.

06 | Early warning signs: notice, do not diagnose

Some children experiencing abuse may show sudden fear of a particular person or place; changes in sleep, appetite, school attendance or concentration; withdrawal; distress after online contact; unexplained gifts or money; or injuries. Others show few outward signs. These changes have many possible causes, so the correct action is a calm, non-accusatory check-in and consultation with an appropriate safeguarding professional, not an assumption that abuse has occurred.

> **Avoid turning warning-sign checklists into public screening tools or asking a child to reveal traumatic details in a group setting.**

07 | Effects on health, learning and development

Violence may affect physical safety, sexual and reproductive health, emotional wellbeing, sleep, trust, school participation, social connection and long-term health. Responses vary; some children may recover with timely support, while others need sustained medical, psychosocial, educational or protection services. Do not tell a child that they will inevitably be permanently damaged.

Support should address practical needs such as safe transport, school continuity, a trusted non-offending caregiver, communication needs, accessibility and freedom from retaliation as well as emotional distress.

08 | Safe first response when a child tells you about abuse

The first adult response can shape whether a child receives help. Use calm, simple, nonjudgmental language. Listen, thank the child for telling you, affirm that the harm is not their responsibility, and explain what will happen next using age-appropriate words.

A helpful first response

- “Thank you for telling me. I am listening.”
- “What happened is not your fault.”
- “You do not need to tell me more details right now.”
- “I want to help keep you safe. I may need to speak to people whose job is to protect children; I will explain what I can.”

What to avoid

- Do not demand a detailed chronological account or repeat the child’s story to multiple people.
- Do not ask leading questions such as “Did your uncle do this?” or investigate yourself.
- Do not promise complete secrecy or confront a suspected offender.
- Do not blame, shame, punish or force the child to publicly identify anyone.
- Do not pressure a child to forgive, reconcile or meet the person accused of abuse.

Specialist interviews, medical examinations and evidence handling should be performed by appropriately trained professionals following child-protection procedures.

09 | Confidentiality and safeguarding responsibilities

Confidentiality is important, but it is not absolute when a child may need protection or reporting is legally required. Before asking for sensitive details, describe the limits of privacy in language the young person can understand. Share information only with authorized people who need it for safety and care. Keep records minimal, secure, accurate and factual; do not circulate images or allegations in messaging groups.

Organizations must verify the current Rwandan reporting rules, the relevant authorities, age-related consent and assent requirements, and internal escalation routes with qualified local legal and child-protection experts. When a caregiver may be responsible for harm, do not automatically send the child back to that caregiver without a safety assessment.

10 | Urgent health needs after sexual assault

A child or adolescent who may have experienced recent sexual assault should be offered prompt evaluation by trained healthcare personnel, even without visible injuries. Emergency care can address injuries, sexually transmitted infections, psychological support and, where appropriate, time-sensitive prevention.

HIV post-exposure prophylaxis (PEP): WHO advises assessing and offering PEP when clinically indicated, as soon as possible and no later than 72 hours after relevant exposure. Emergency contraception: WHO guidance describes offering it to eligible girls after rape as soon as possible, within 120 hours (five days), when appropriate. These are clinical decisions for trained providers; the handbook does not instruct readers to self-medicate.

A child who comes for help after these time windows still needs and deserves healthcare, protection and psychosocial support. Do not withhold care because evidence was not preserved or because the family has not made a police report.

11 | Safer school and youth-program responses

Schools and organizations should adopt clear staff conduct rules, an accessible reporting route, trusted designated safeguarding leads, safe transportation and supervision policies, and a prohibition on retaliation. Reporting arrangements must work for children with disabilities and for children who do not speak the primary school language fluently.

Educators should teach respectful relationships, non-discrimination, consent-related body safety and digital skills without asking students to share personal histories. A teacher receiving a disclosure should follow the safeguarding process rather than conduct a private investigation.

12 | Online grooming, harassment and image-based abuse

Digital abuse may involve persistent sexual messages, pressure for intimate images, pretending to be a peer, threatening to disclose private material, impersonation or location tracking. Even if an adolescent voluntarily sent an image, threats and non-consensual distribution are not justified. Never blame the young person or ask them to forward intimate material to an educator or chatbot.

If an offender can access the child’s phone or accounts, sudden changes to passwords or settings may create additional danger. Seek help from a trusted safeguarding or digital-safety professional and use a safe device when possible. Platforms may offer abuse-reporting options, but reports should be made with attention to child safety and local procedures.

13 | Prevention: family, community and institutions

The WHO/UNICEF INSPIRE framework identifies seven complementary strategies: implementing and enforcing laws; changing harmful norms and values; creating safe environments; supporting parents and caregivers; strengthening household income and economic security; providing response and support services; and improving education and life skills. Child protection cannot be achieved by putting all responsibility on individual children.

At home, caregivers can use respectful communication, nonviolent discipline, predictable support and age-appropriate discussions about boundaries. In the community, leaders can challenge narratives that blame survivors or shield powerful adults. Institutions should create accountable reporting channels and transparent procedures.

14 | Access to support in Rwanda

Rwanda's Ministry of Gender and Family Promotion (MIGEPROF) describes Isange One Stop Centres as providing coordinated healthcare, psychosocial, justice and protection services for GBV and child abuse. UNHCR Rwanda lists the Isange toll-free number as 3029. MIGEPROF publishes 711 for the National Child Development Agency (NCDA) and 116 / 3512 for the Rwanda Investigation Bureau (RIB). Availability and suitability should be checked at the time of use.

UNHCR notes that approaching an Isange Centre may result in a report to law enforcement. Before sharing sensitive details, seek an explanation of privacy, safeguarding and reporting requirements. If a child is in immediate danger, contact an appropriate emergency or child-protection service without delay.

15 | Supporting different needs

A child with a disability may need a qualified interpreter, accessible transport, visual communication or more time to express concerns. Avoid relying on a suspected offender as an interpreter or caregiver during a disclosure. Children in displacement, migration, institutional care or severe economic hardship may need integrated practical support. All children should be treated with equal dignity.

16 | Fictional, Rwanda-relevant learning scenarios

Scenario A — Secondary school

A student becomes uncomfortable because a staff member repeatedly sends private late-night messages and promises special marks if the student keeps the relationship secret.

Learning response: Possible power misuse and grooming indicators. The student should not be blamed. A trained safeguarding contact should respond and preserve privacy.

Scenario B — Household dependence

A young adolescent fears returning home because an adult caregiver makes threats and says nobody will believe them.

Learning response: Immediate safety and a trusted non-offending adult matter. Do not confront the caregiver or send the adolescent back without an appropriate safety assessment.

Scenario C — Phone-based coercion

A teenager is threatened with distribution of a private image unless they send additional images.

Learning response: Treat as digital coercion. Avoid requesting the image itself. Consult safe adult support and child-protection pathways; assess device monitoring.

Scenario D — Community sports

A coach insists on private unsupervised meetings and asks an athlete not to tell their family.

Learning response: Boundary concerns warrant safeguarding review. Organizations should prevent one-to-one secrecy and make reporting accessible.

Scenario E — Friend disclosure

One adolescent tells a friend that an adult has been touching them in ways that feel unsafe.

Learning response: The friend can listen, avoid interrogation, and connect the child to a safe adult or professional; a peer should not be made solely responsible for safety.

17 | Guided educational activities

Activity 1 — Map a safe support network

For educators and caregivers: identify three roles, not named children: a trusted safeguarding lead, a trained health/service provider and a responsible protection contact. Verify the route between them.

Activity 2 — Practice supportive phrases

In pairs, practice a response to an invented disclosure using four principles: listen, believe without investigating, explain confidentiality limits, and refer.

Activity 3 — Spot misuse of authority

Read Scenario A. Identify the authority imbalance, secrecy request and possible benefit offered. Explain why the student bears no responsibility for the adult’s conduct.

Activity 4 — School safety audit

Check whether an institution has a child-friendly complaint method, disability access, retaliation protection, staff boundaries and documented referral procedures.

Activity 5 — Digital safety choices

Read Scenario C. List three steps that require safety assessment before acting: blocking, password changes and collecting digital evidence. Do not use real images or accounts.

18 | Knowledge assessment

1. Does a child need visible injuries before adults should take a disclosure seriously?

Suggested answer: No. Many forms of abuse leave no visible injury.

2. Should a teacher ask a child to repeat every detail to prove the account?

Suggested answer: No. Avoid repeated questioning and refer to trained professionals.

3. Can a child’s silence or receipt of gifts demonstrate sexual consent?

Suggested answer: No. Power, age, fear and dependence undermine such claims.

4. Is every behavioral change proof of GBV?

Suggested answer: No. Warning signs have many possible explanations; respond carefully.

5. What should a first responder say about secrecy?

Suggested answer: Explain confidentiality and its safeguarding/legal limits honestly.

6. Does the 72-hour PEP window mean a late-presenting child needs no care?

Suggested answer: No. Other medical, psychosocial and protection care remains important.

7. Should staff forward an adolescent’s intimate image in a group chat for evidence?

Suggested answer: No. This can increase harm; use designated professional procedures.

8. Is safety planning only the responsibility of the young person?

Suggested answer: No. Responsible adults and institutions must protect children.

9. What is the purpose of INSPIRE?

Suggested answer: To guide multisector prevention of violence against children.

10. Why verify referral contacts and legal rules before publication?

Suggested answer: Service availability and local procedures can change; inaccurate advice may increase risk.

19 | Key terms

Child safeguarding: Organizational and professional duties to prevent and respond to harm against children.

Disclosure: When someone communicates information or concerns about abuse.

Grooming: A pattern of manipulation and boundary crossing intended to facilitate exploitation.

Coercion: Pressure, intimidation, threats or misuse of power that undermines free choice.

Non-offending caregiver: A caregiver not implicated in the harm who can support safe care.

Trauma-informed care: An approach that prioritizes physical and emotional safety, dignity, choice and avoiding re-traumatization.

PEP: Time-sensitive medication prescribed by a clinician after qualifying HIV exposure.

Referral pathway: An agreed process for connecting someone with appropriate specialist support.

20 | Source notes and references

WHO (2017), Responding to children and adolescents who have been sexually abused
https://www.who.int/news/item/19-10-2017-responding-to-children-and-adolescents-who-have-been-sexually-abused

WHO (2016), INSPIRE: Seven strategies for ending violence against children
https://www.who.int/publications/inspire-seven-strategies-for-ending-violence-against-children

UNICEF, INSPIRE Handbook
https://www.unicef.org/documents/inspire-handbook-action-implementing-seven-strategies

WHO (2024), Guidelines for HIV post-exposure prophylaxis
https://www.who.int/publications/i/item/9789240095137

MIGEPROF, Directorate General of Family Promotion and Child Protection
https://www.migeprof.gov.rw/family-promotion-and-child-right-protection

UNHCR Rwanda, Gender-Based Violence - services and referral
https://help.unhcr.org/rwanda/services/sgbv/

WHO clinical guidelines, detailed recommendations for sexually abused children
https://www.ncbi.nlm.nih.gov/books/NBK493107/

Publication and safeguarding review

- A qualified Rwanda child-protection or GBV specialist has reviewed every example and safety statement.
- A qualified clinician has reviewed clinical content, including PEP and emergency contraception.
- A qualified Rwanda legal/safeguarding reviewer has verified reporting duties and confidentiality limits.
- All phone numbers, Isange access details and referral procedures have been checked again immediately before release.
- Kinyarwanda and French versions have undergone human localization and independent review.
- The publication does not solicit personal disclosures, identify real children or contain unsafe image-handling instructions.

STATUS: Evidence-informed educational draft. Not approved for direct use as a child-protection service protocol or personalized clinical advice.